

I: Yeah.

I: Okay, so then let's start with the actual questions.

I: Okay.

I: The first question is familiarity with AI.

I: How familiar are you with AI technologies in general?

I: uh it's like a score that you like so so there are two questions with the score but okay uh so this one is one of them but i also want like open just talk about like how you

P: okay like i'm familiar with AI but in portugal we don't use it much okay we you we know the the

P: the concept okay but we don't use it for like a practical in the in the in the interview or in the evaluation or the first time you we use like more like a tool in a very um psychoeducation way i'm familiar so if we we have to score i think it's eight in the scale of

P: zero to 10 I don't know what did you say eight yeah yeah because I'm I'm familiar with that but not I don't know because of my country and the way that people see um psychological treatment is like very to one to one you know like it's very important to

P: to establish like a to very early alliance.

P: So and it's a very therapeutic relationship.

P: So many people, for example, it's not either for example, the online like interviewing or counseling.

P: It's only began in Portugal, because the COVID.

P: So it's very,

P: Sorry for my English.

I: So it's very like focused on human to human connection and the online interactions have only started with COVID.

I: So because people can see each other, they started using the online e-health, right?

P: Yeah.

P: And for example, I don't know if I can explain a little, but for example, I work in a hospital, there is cancer.

P: and every people have cancer, okay?

P: And for example, my work is on palliative care.

P: So it's very difficult to use AI.

P: For example, we only use AI in a virtual way.

P: For example, the people or I have a patient that have a dream to travel to Australia.

P: And we can put like the, I don't know how to say, like the glasses, you know?

I: Like the VR headset?

P: Yeah.

P: And we manage just to check the list of the, I don't know how to say, for example, they have

P: um like a list that he wants to accomplish uh before he's dead it's just like a like a to-do list or like uh yeah yeah like i want to do this is the way that only i use AI on my

P: on my doing

I: yeah okay

I: um okay so is that something that your organization introduced themselves so has your organization introduced any ai tools or systems

P: um yeah i i think we can we can say yes

I: um

P: Like, for example, we do a platform.

P: We have a platform that, for example, people have...

P: It's more for breast cancer, okay?

P: And we have, like, for example, people ask or do a form that responds to...

P: It's like a score or a questionnaire about how do you feel post-surgery and if self-love or relationships with husband or wife, in this case, husband, and for the response that the

P: our patients have we like the platform give give them like flyers or contacts, for example, for my units, the psychological units, and they can call us, but it's like this point of view, not much.

P: So only for breast cancer.

I: yeah yeah so so they get some kind of questionnaire and then so how is ai used in that

P: um is for example um the the score they score for for example um a medium of everything and after um it's uh i think they have a question that um

P: um so cycle um our cycle is notifiable you want to chat a little bit or do you need some uh advice or you need something and do they pop up like something

I: So there is like a questionnaire and this questionnaire, based on how you respond on this questionnaire, there is some AI that's analyzing your responses.

I: And then based on your responses, it might say, oh, here's a flyer about whatever.

I: Or here is some contact information for psychologists that we are connected with.

P: And this flyer is a flyer that we do and we put there.

P: Or, for example, some formations that I don't know how to say because it's a Portuguese thing.

P: It's like Liga Portuguesa Contra o Cancro.

P: It's like an associate that gives some formation or some educational behavior for people with cancer.

I: Yeah.

P: And we, yeah.

I: Okay.

I: But then when, when you use AI, um, your clients are also aware that you are using AI or did you not tell them that?

P: No, no, we, we tell them because like, it's like, um, a consent.

P: Okay.

P: we form it for example when the the flyers appear we tell that so when the formulary and the things appear in the beginning we tell them that the response can generally suggest some things that AI are gonna

I: So there will be like a pop-up that says your responses will be checked by AI?

P: Because it's an ethical thing that we have to do on our hospital.

I: I mean, I just have to check, right?

I: Because sometimes people are actually not doing it.

P: But really, okay, we have to do because we have like an ethical to do this, this platform, we have to do like, ethical things in the in the hospital.

P: And for example, the people before respond to the formally have like a consent that the things that suggest before they respond to the formally are generate from like our supervision, but it's create like for the response that they gave.

I: Yeah.

I: Makes sense.

I: Then we can move on to the next question.

I: This is about diagnostic and assessment use.

I: So what are your thoughts on AI being used in diagnostic or assessment processes?

P: Yeah, that is a tricky question because it's like I was saying, like AI for me is like a tool.

P: that you use like a tool, not for a thing that you can manage it.

P: And, okay, their response, for example, their response, okay, she have like depression or anxiety, okay?

P: It's like a tool that, okay, she respond to that.

P: And for my intrepidity, for my judgment, for my knowledge of something, I, okay, but it's like a tool for me.

I: Yeah.

I: So you think AI is more of a tool instead of like an actual thing that you can base your judgment on, right?

I: Like you still have to make your own judgment as a clinician to come up with your own judgment and not just say, oh, what does the AI say?

I: It says this person has anxiety disorder.

I: We accept, right?

P: Yeah, for example, my in my private work, I have some some patients that I I tried to to write and or sell in the Google or chat GPT.

P: And what is vulnerability?

P: Because you you

P: tell me that vulnerability is a thing that we can have to work on it, okay?

P: And sometimes it's very difficult for me, like a psychologist, because it's like a risk and sometimes it's educational, depends on the way that the patients search for the...

P: the things but yeah i think it's a tool but not that a thing that you can okay she scored seven seven it's like very anxiety so she's a um an anxiety problem or like a depression or a comorbidity depression or i don't know yeah

I: Yeah, so you mentioned some example where a client of yours, they checked with ChatGPT.

I: No, you talk too much.

I: No, no, no, that's totally fine.

I: I would just maybe summarize sometimes to check if what you're saying, if my understanding of that is correct, right?

I: So you mentioned that there was an example, right, that someone used ChatGPT to check what vulnerability was because that was something that you mentioned in the treatment that they should work on that or like that you're working on that.

I: And there is some risk there that sometimes this information that ChatGPT will give might not be correct.

I: But there is also opportunity because sometimes it might be correct or it might be helpful for that person to figure out, oh, this is vulnerability, right?

I: But you also say that, so it could be used as a tool like this, but it could not be used in a way where if someone

I: has a result of seven on some scale this means that they have this disorder right yeah yeah okay that makes sense um so do you do you currently do any intake processes

I: um for give me an example sorry so for example the first usually in in psychology you have like a first session where you where you talk with your client and ask them about their problems yeah um so this is called like the intake right so do you do an intake with your clients yeah yeah and do you think ai could help in this intake

P: I think no.

P: For the first time, no.

P: Because how to explain?

P: Portuguese people, for example, I have a patient that is not Portuguese and is very mind open about the AI or something like

P: that we more creative something but in the in the in my opinion i think the first and the first um sessions are very important to establish like a uh to perica license so if you put a tool um in the first session uh like the the the way that i see AI i think it's broken

P: I think the relationship are not doing in the fluid way, you know, I don't know how to, I don't know if it's understandable that I'm saying, but I think it's, for example, in the third or fourth session, I think it's, for example, when you start with exercise or something that can help

P: people or patients monitoring something, for example, triggers that, for example, anxiety.

P: I used...

P: very monitoring assessment or exercises to people notice the triggers, how it starts, what thoughts that I brought, when it starts, what I think, how I feel, what's the situation.

P: It's like more in a cognitive structure, okay?

P: But in the first sessions, no, I don't use.

I: Yeah, so you think the first session is really important to start working on that therapeutic alliance, right?

I: Yeah.

P: And that you think... Because if you think it's... Like, for example, when you choose to start...

P: a therapeutic process, you are very nervous because sometimes you don't know what to, what can I start, what I'm going to say, are they judge me?

P: My patients talk to me after the first and tell, oh, when you know me, what do you think?

P: What do you feel?

P: So if I give them a tool and not a person,

P: I think they're going to be defensive, you know, and so for me, it's not the thing that I use in the first place.

I: yeah so you think that uh the first sessions are very important to start on that alliance and if you introduce some ai system to that then that will definitely block some of that openness and people might become like defensive or um you also mentioned that they they ask you what your judgment is on their situation and how they are speaking or their problems and

I: You think that introducing AI in that will make it so they will be less open, right?

I: Yeah.

I: But you say that you could maybe use it later in like the fourth or the fifth or whatever, like later in the treatment.

I: If you are already starting to do some exercises, for example, if you start doing CBT or whatever, like...

I: There are specific treatments that sometimes maybe you can use it in explaining exercises or doing the exercises with some AI.

P: Yeah, because the first sessions you know the person.

P: You understand what's important for that person, what are the things that matters to that person.

P: And after

P: I would say you have a tool that, okay, I'm a psychologist, I know the person, but sometimes the tool that I have cannot match.

P: And AI is a tool that sometimes can give you like a connection that what you know the person, know the tool that you want to give them, but sometimes you have to make personalized, you know?

I: Yeah, so also with those first sessions, you figure out how this person might react to AI and you maybe need to adjust this AI to make it fit better or maybe it doesn't even fit at all.

I: And then you can decide, okay, I don't think this tool will be useful for you, right?

I: Yeah.

I: Okay, there's one more follow up.

I: So have you considered AI for preliminary screening?

I: Do you know what preliminary screening is?

I: Yeah.

I: So this is like the screening before the intakes, right?

I: And you kind of mentioned that you already do some questionnaires where AI is being used, where they send them to you, right?

I: So can I assume that you are already using AI for preliminary screening?

P: Yeah.

P: In my public work, yeah.

P: Because it's like a thing that we have in hospital.

P: So me and my colleagues have to, yeah.

P: use in the first in the first time because it's not only psychologists that have access doctors and nurse have too and we combine like the symptoms and the script that the response have and we talk to each other to okay

P: this person maybe it's better to have like an appointment of psychologist like the next week or the doctors have to talk about for example I don't know how to say but it's like a surgery that's a statical surgery I don't know for example okay but it's like I think things that we manage the team

I: Okay.

I: Makes sense.

I: Then I will move on to the next question.

I: So this is about monitoring and progress tracking.

I: How do you view the role of AI in tracking patient progress over time?

P: I think I, I respond to that question a little bit, but, um,

P: I think it's very, for example, when I use it more on anxiety or for example, to people know how they feel in similar situations.

P: Or for example, the one that I use most right now is for sleep.

P: have some exercise that for example or the watch that the people have to use during the sleep and you monitoring the cardio rhythm and the hours of sleep and if it sleeps well what do you feel in the all of the day is the one that I'm doing some

P: things right now but yeah yeah so you are are using some smart watches right for for tracking sleep and like cardio uh yeah and anxiety too because it's very yeah three things that are very combined so um is the things that uh right now i'm on everything the most yeah um

I: Uh, so, so that's, that's kind of the scope in which you think that AI could help in monitoring, right?

I: Yeah.

I: Yeah.

I: Um, yeah.

I: Uh, what concerns, if any, would you have about AI tracking patient progress?

P: Um, I think it's, um,

P: It's like the thing that I told you recently.

P: It's like, for example, they told that, for example, I have a big dream last night, okay?

P: It happens like I have a meeting before you with a patient and he told me that, okay, I'm the...

P: in the Saturday or Sunday, I don't know, and he told me, okay, I do sports, I run, I have time with my family, things that she wants and it's okay.

P: But for example, the sleep or the

P: the society scores very high.

P: She feels very, very uncomfortable in the weekend.

P: And we are talking like the monitoring, what do you feel, what do you... And she told me, I'm going to chat GPT and like doing the question.

P: Is something wrong with me?

P: Am I doing everything right?

P: And why is... And this is the risk because we are doing a process, you know?

P: And for example, I tell them, if you need anything, please send me a message or if you need, please call me and we...

P: talk a little bit and she preferred to write the question and today she told me I might be useful to talk to you because what I see it's not what we are doing okay and I think and this session is like from our self-love

P: And we have to recognize the concepts and accept that in a little times we don't have like a lot of response or the response that you want, but we have to do like the progress and you do the way to getting there.

P: But sometimes you have like one step, two steps behind, one step, two steps behind.

P: So this is, sometimes this is the risk to, because it's very, for example, if she called me and I don't answer, she can like always write a question and ChatGPT like response.

P: But sometimes it's not personalized.

P: It's not, the chat don't know the person who are questioning.

P: So the things are very difficult sometimes to,

P: The person has the time to, okay, this is rational, okay, this makes sense, but for me, okay, it does not make sense.

P: But sometimes you cannot do it.

P: So it's very difficult on the process to do this.

I: Yeah, so one of the possible concerns is that with the progress tracking or in general, just monitoring and tracking, if your client becomes anxious or wants to know something about how it's going, maybe their sleep score is bad that day or their anxiety score is bad that day.

I: If they then immediately send to ChatGPT, oh, is there something wrong?

I: Am I doing like what is going on?

I: Then there's a possibility that

I: ChatGPT will not know the full context of this person and also not the full context of the treatment plan.

I: And then they might give wrong answers or answers that are not helpful in reducing that anxiety, right?

I: Where instead of them asking ChatGPT, you would prefer them to ask you because you are a professional, right?

I: So you also know their context.

I: um and you could just tell them okay well sometimes you will just have days where you have worse sleep or you have worse anxiety and that's fine but we will slowly in this pro progress or like process we will slowly move from the situation we were in we might move two steps forward and then one backwards right sometimes you have to go back and forth but

I: You just kind of have to trust the process and there is a chance that patients will use AI systems to increase their anxiety because they want to check how it's going, right?

I: Yeah.

I: Okay.

I: Then for the next question, this is about administrative and practical support.

I: How do you feel about AI generating drafts or reports or assisting with scheduling and reminders?

P: yeah um i don't use AI for that so um it's me doing uh but but i can see a potential a potential to for example um the i the AI create for example send the email or send the text for the a new um

P: meeting or new session for us to have more time to, for example, prepare the meeting, prepare some exercise to do.

P: But I prefer to do it on my own.

I: Wait, you prefer to do it yourself?

P: Yeah.

P: Yeah.

P: For example, I have a

P: I have my clients and the day before I send them the message that don't remind that tomorrow at 7 a.m. or 7 p.m. we have a meeting.

I: So you prefer sending that email yourself or you would be open to AI doing that for you?

P: I'm be open, but right now I sense.

I: Yeah.

I: Yeah.

I: Okay.

I: Makes sense.

I: Yeah.

I: Um, so how much time do you think this could save you?

I: Um, a lot.

I: Can you give an estimate?

P: Uh, I don't know, but for example, uh, tomorrow I have, uh, 10 patients on, uh, yeah, no,

P: 15 so if for example if i don't know but one hour massive that i have more if i don't send them uh reminders or things yeah like one hour a day maybe yeah yeah okay that makes sense um

I: let's move on to the next question this is about psychoeducation and between session tools um what are your thoughts on using ai for psychoeducation or therapeutic exercises between sessions

P: yeah um i think it's okay i use only um respond to that but sometimes it's very uh

P: not me, but friends of Maldarraça College to use them to create some following on Instagram or in the social networks.

P: For example, you have, you want to do like a publishing of people with cancer and you have an idea and they

P: talk to me to oh uh i have an idea to do a publish on instagram to people of cancer and the psychological uh impact uh but i know i don't know how to put it so i asked to AI to help me to structure a thing for example

P: I don't do that, but I understand that sometimes for the creativity or the structure, it helps.

P: I do it more to, like, educational, to do exercise or monitoring, but I don't do it a lot.

P: For example, I think it's maybe...

P: bad to tell it but for example the people that I have that for example patients that are engineering okay to have more I don't know how to say but it's like more

P: engagement to the process okay um sometimes they um be engaged if i tell them i always tell them but if i tell okay let's do this i do this with the AI and oh okay it's very nice it's very okay and they engagement into the process yeah

I: Yeah, could you, so, okay, maybe let me summarize a bit.

I: So you mentioned that some of your colleagues have used it for social media content creation, right?

I: When they want to make a post about like cancer and some education on that and maybe the psychological effects of cancer.

I: So they ask some online or like AI system and they will help them with their creative side of things, right?

I: And then also you mentioned that you do use it maybe sometimes for exercise and monitoring,

I: and you for example uh also mention it or like you do mention it every time you use it but you do uh explicitly um use it maybe more with for example engineering uh focused people because they are more excited about ai stuff so

I: to in order to make them more engaged in treatment you introduce ai so they are more engaged right yeah could you maybe explain some of the like like an example of exercise you make with ai uh it's difficult because my english it's not very very good so i'm gonna try

P: um for example you have like um i don't know uh how to or i don't know if you know but it's like um a thing that uh exists that it's like uh um for example the people uh it's like it's online i can send you but it's like a

I: Oh.

I: Hello, hello, hello.

I: I'm sorry, I think the connection is cutting out a bit.

I: Hello?

I: Hello?

I: Are we back?

I: Okay, I think it cut out a bit.

I: So I didn't get any explanation.

P: Okay, for example, we have like, I think that some

P: uh colleagues uh introduce me it's like wiser w y okay that it's for example you have a thought you have a feeling and they put on a motion for example from very uh inside people is very difficult to know how how they feel with anxiety

P: because the thoughts are very um don't stop okay the thoughts are constantly over and over and over and over yeah so we put it on the thought we have like an anxiety and they understand i don't know how to explain this a little bit because my english is not very very very okay but um

P: they put in a in a emotion or it's a feeling and have like um exercise that can help you to reduce the anxiety for example uh breathe um like um um i don't know how to say in english but i'm gonna try for example uh to put you on the present moment we do like a ground ring um

P: exercise that for example with your breathing okay we ask my patients to okay uh think to five things that you see right now four things that you hear right now um or three things that you can touch okay and then they do the exercise i can't i don't have to be with them

P: But after they told me, okay, I do this exercise.

P: Okay, what do you feel?

P: The anxiety, it's better, it's not better.

P: You feel anything else and you have like a supervision for the practice.

I: I don't know.

I: So where does AI come into this?

I: Like the exercises, don't they already exist?

P: yeah but for example a person that's have like um um i don't know have like um a symptom and

P: express to the the the exercise that okay i'm feeling this my anxiety is overload please for example help me and this please help me uh have like a

I: exercise of breathing exercise like the okay so let me check if i understand so there is like this type of exercise that you do and it's for example with anxiety patients right you have that their thoughts are like racing because they just have anxiety coming in the whole time and they're not even sure how they are feeling but then there is some grounding exercise that you could do

I: and getting this grounding exercise they get through ai right they just say oh i'm feeling this or i'm being very anxious and then the ai will provide um this exercise some grounding exercise for example think of five things around you like point them out think of four like whatever like that's kind of grounding right um and because their attention gets like uh forced onto like ex

I: internal objects, it kind of goes away from their anxiety.

I: So then afterwards they will check how is my anxiety now and most likely it will be lower.

I: And that's kind of the exercise.

I: Right.

I: But the AI is just giving exercises to the patient that's just asking for it.

I: Right.

I: Yeah.

I: Okay.

I: That makes sense.

I: How are you on time?

I: sorry how is your time because no it's okay okay okay because we still have a few questions but we can okay um uh we can end early if you really need to go but i of course want to hear like 10 10 10 minutes if it's okay yeah okay that's fine

I: then I will cut you off faster if there's something that's like been discussed already.

I: So next question, risks and limitations.

I: What risks or concerns do you have about using AI in clinical work?

I: So you mentioned already a few, right?

I: where you think you need to get the therapeutic alliance going.

I: So it's like losing the trust.

I: And also sometimes AI systems might give like wrong information or not helpful information because they don't have the context of the specific client, right?

I: Is there any other limitations or risks that you want to mention?

P: I don't know.

P: I think it's the only thing it's like...

P: it's like a tool.

P: I don't know how to explain.

P: Because sometimes it's very difficult to talk about it because I don't know to being rude about the team, but it's like it's not like replace a human, you know?

P: And sometimes I think people think the AI can replace a human and they don't.

P: In very...

I: very uh otherwise but in a psychologist way i cannot imagine this is happening so it's the risk that uh i'm very concerned sometimes yeah so then what you mentioned is that ai is not going to replace psychologists right it will it will it can be used for as a tool but it will never be

I: It's never going to be the case where a patient or a client is talking directly to AI and there's never someone on the other side.

P: Because it's not like an interpretation.

P: It's like human.

P: You like feelings to talk about feelings.

P: It's a human connection.

P: Like what I was saying, the first sessions are very important to establish a relationship.

P: And if you are talking with a machine, sometimes it's very difficult to...

P: to try this way.

P: And sometimes I don't know why I have afraid like, the machine gets too real for some patients, you know, it's like, it's not human, but from patients, it's gets very real to talk with the chefs.

P: That's sometimes my, my, I don't know, I have afraid that people

P: don't understand that it's a machine that you can replace for humans.

I: Do you think there's a possibility of some clients to become over reliant on AI?

I: Yeah.

I: Like they just get this idea that AI is some person or maybe not even that, but like they become so engaged.

P: Yeah.

P: People like adoption.

P: Yeah.

P: Yeah.

P: Because

P: For example, they told me in the session that, okay, I have a doubt, I question ChatGPT.

P: I don't talk to my mom, I don't talk to my father, I talk to ChatGPT.

P: And it's very horrifying to be a psychologist in that way.

P: But

P: i don't know it's a i'm very scared about about it so i think this is why i it's very difficult for me to see uh with good eyes the

I: yeah makes sense um okay then for the next question this is about the client perspective and therapeutic alliance so i think we talked about this a lot yeah but

I: How do you think clients respond to AI being used in their treatment?

I: You don't think it is that good to introduce it, right?

I: And you also think there is a chance for them to become over reliant on it.

I: Yeah.

I: Could you also score how do you think clients respond to AI being used in their treatment?

I: So one being they will not like using AI and 10 being that they would like it.

P: Okay.

P: For example, my clients that are middle age, for example, like 30 or 35, I think they are okay with it.

P: Like, for example, a six or seven.

P: But if they are older, no, they refuse.

P: For example, even exercise, I have to do it in sessions.

P: I can do it, for example, to homework.

P: I have to do it in session for older people.

P: For middle age, I can do it on home and after we supervise in a session.

I: Yeah, so you think younger people are more open to it, so maybe like a six or a seven, but then the older people might not be that open to it.

P: Yeah, in Portugal, because for example, the clients that I have older and they are not Portuguese, they are very mind-open to do things like with the AI or they talk to me about AI and other things because, for example, I have a patient that

P: um love to travel alone and he told me in the in the session that okay i plan with the help of chat gpt um three days in the somewhere okay so yeah uh so could you give maybe an average score on like whole population i don't know um

P: I think six.

I: Okay.

I: Okay, then this is the last question.

I: Okay.

I: This is about professional identity, autonomy and future conditions.

I: So this is actually like a two part question.

I: Which aspects of your work should remain strictly human?

I: And what would you need to feel confident about using AI safely in practice?

P: Okay.

P: I think like, um, like our evaluation, our, um, um, I don't know how to say it, like assessments or for example, to, um, yeah.

P: Uh, but, but in Portugal we don't say like diagnosis.

P: We say like, for example, we, you have like, um,

P: a disorder, okay?

P: We do like an assessment, a doc, and you give him to doctor or at work to have some benefits with it, okay?

P: Yeah.

P: And I think, I don't know, but it's a tricky question for me because I have

P: concept of AI that I told you that make me very concerned to only think about it.

P: But I don't know, maybe see more people using it and have like studies that confirms or like

P: I don't know, but it's like limited.

P: I don't know.

P: I think I don't, I don't have like, I don't know.

P: It's a very tricky question.

P: Because I think like, what I do, only human can do.

P: Yeah.

P: Like, talk to person, feel what your person feel.

P: And be impacted, be there for the person.

P: It's not a yeah, yeah, doing.

P: So it's very difficult for me to understand.

P: But I know, for example, the things that I talk to you, like, for example, the schedule, the organized, for example, my agenda or like a tool.

P: Yeah.

P: But I don't know how to...

I: I will summarize a bit, okay?

I: So what should remain strictly human?

I: You said the relationship, so the assessment should also just be human because you think that that's really important and it's all based on the therapeutic alliance, right?

I: And then for you to be confident in using AI, it's quite hard for you to imagine this because your general opinion of using AI

I: is not that high where you don't think it could really be used in a way where it's replacing the human, right?

I: But if there is something that you should mention, then maybe that's just that

I: that more people are using it, that there's more scientific proof of that it works or whatever, then maybe you would consider using it more, right?

I: But still you think this is not going to be the case because the practical and the actual real world scenario will just be that you still have to talk with the client.

I: And that's kind of it, I think, no?

P: Yeah.

P: It's more like an ethical way to think about it.

P: For example, the concept of ethical is very important on my professional.

P: So it's very difficult to see AI as ethical.

P: It's a privacy concept.

P: It's very difficult for me to, because the risk of privacy, the link of privacy, the notes, for example, are, I don't know, it's very terrifying for me.

I: Okay, makes sense.

I: So, I think we're done.

I: You don't want to add anything?

P: No, just to say sorry for my English.

P: No, that's okay.

P: That I can.